

Part B

In this part of the test, there are six short extracts relating to the work of health professionals. For questions 1-6, choose answer (A, B or C) which you think fits best according to the text.

1. The policy document tells us that

- (A) stop dates aren't relevant in all circumstances.
- (B) anyone using EPMA can disregard the request for a stop date.
- (C) prescribers must know in advance of prescribing what the stop date should be.

Prescribing stop dates

Prescribers should write a review date or a stop date on the electronic prescribing system EPMA or the medicine chart for each antimicrobial agent prescribed. On the EPMA, there is a forced entry for stop dates on oral antimicrobials. There is not a forced stop date on EPMA for IV antimicrobial treatment – if the prescriber knows how long the course of IV should be, then the stop date can be filled in. If not known, then a review should be added to the additional information, e.g. 'review after 48 hrs'. If the prescriber decides treatment needs to continue beyond the stop date or course length indicated, then it is their responsibility to amend the chart. In critical care, it has been agreed that the routine use of review/stop dates on the charts is not always appropriate.

2. The guidelines inform us that personalised equipment for radiotherapy

- (A) is advisable for all patients.
- (B) improves precision during radiation.
- (C) needs to be tested at the first consultation.

Guidelines: Radiotherapy Simulation Planning Appointment

The initial appointment may also be referred to as the Simulation Appointment. During this appointment you will discuss your patient's medical history and treatment options, and agree on a radiotherapy treatment plan. The first step is usually to take a CT scan of the area requiring treatment. The patient will meet the radiation oncologist, their registrar and radiation therapists. A decision will be made regarding the best and most comfortable position for treatment, and this will be replicated daily for the duration of the treatment. Depending on the area of the body to be treated, personalised equipment such as a face mask may be used to stabilise the patient's position. This equipment helps keep the patient comfortable and still during the treatment and makes the treatment more accurate.

3. The purpose of these instructions is to explain how to

- (A) monitor an ECG reading.
- (B) position electrodes correctly.
- (C) handle an animal during an ECG procedure.

CT200CV Veterinarian Electrocardiograph User Manual

Animal connections

Good electrode connection is the most important factor in recording a high quality ECG. By following a few basic steps, consistent, clean recordings can be achieved.

1. Shave a patch on each forelimb of the animal at the contact site.
2. Clean the electrode sites with an alcohol swab or sterilising agent.
3. Attach clips to the ECG leads.
4. Place a small amount of ECG electrode gel on the metal electrode of the limb strap or adapter clip.
5. Pinch skin on animal and place clips on the shaved skin area of the animal being tested. The animal must be kept still.
6. Check the LCD display for a constant heart reading.
7. If there is no heart reading, you have a contact problem with one or more of the leads.
8. Recheck the leads and reapply the clips to the shaven skin of the animal.

4. The group known as 'impatient patients' are more likely to continue with a course of prescribed medication if
- (A) their treatment can be completed over a reduced period of time.
 - (B) it is possible to link their treatment with a financial advantage.
 - (C) its short-term benefits are explained to them.

Medication adherence and impatient patients

A recent article addressed the behaviour of people who have a 'taste for the present rather than the future'. It proposed that these so-called 'impatient patients' are unlikely to adhere to medications that require use over an extended period. The article proposes that, an 'impatience genotype' exists and that assessing these patients' view of the future while stressing the immediate advantages of adherence may improve adherence rates more than emphasizing potentially distant complications. The authors suggest that rather than attempting to change the character of those who are 'impatient', it may be wise to ascertain the patient's individual priorities, particularly as they relate to immediate gains. For example, while advising an 'impatient' patient with diabetes, stressing improvement in visual acuity rather than avoidance of retinopathy may result in greater medication adherence rates. Additionally, linking the cost of frequently changing prescription lenses when visual acuity fluctuates with glycemic levels may sometimes provide the patient with an immediate financial motivation for improving adherence.

5. The memo reminds nursing staff to avoid
- (A) x-raying a patient unless pH readings exceed 5.5.
 - (B) the use of a particular method of testing pH levels.
 - (C) reliance on pH testing in patients taking acid-inhibiting medication.

Checking the position of a nasogastric tube

It is essential to confirm the position of the tube in the stomach by one of the following:

- Testing pH of aspirate: gastric placement is indicated by a pH of less than 4, but may increase to between pH 4-6 if the patient is receiving acid-inhibiting drugs. Blue litmus paper is insufficiently sensitive to adequately distinguish between levels of acidity of aspirate.
- X-rays: will only confirm position at the time the X-ray is carried out. The tube may have moved by the time the patient has returned to the ward. In the absence of a positive aspirate test, where pH readings are more than 5.5, or in a patient who is unconscious or on a ventilator, an X-ray must be obtained to confirm the initial position of the nasogastric tube.

6. This extract informs us that

- (A) the amount of oxytocin given will depend on how the patient reacts.
- (B) the patient will go into labour as soon as oxytocin is administered.
- (C) the staff should inspect the oxytocin pump before use.

Extract from guidelines: Oxytocin

1 Oxytocin Dosage and Administration

Parenteral drug products should be inspected visually for particulate matter and discoloration prior to administration, whenever solution and container permit. Dosage of Oxytocin is determined by the uterine response. The dosage information below is based upon various regimens and indications in general use.

1.1 Induction or Stimulation of Labour

Intravenous infusion (drip method) is the only acceptable method of administration for the induction or stimulation of labour. Accurate control of the rate of infusion flow is essential. An infusion pump or other such device and frequent monitoring of strength of contractions and foetal heart rate are necessary for the safe administration of Oxytocin for the induction or stimulation of labour. If uterine contractions become too powerful, the infusion can be abruptly stopped, and oxytocic stimulation of the uterine musculature will soon wane.